AI HEALTH WEEKLY

What the week's evidence supports, and what it does not.

THE PRESENTING WEEK

The week the rules diverged

Washington erased the AI label from health-record certification; the WHO answered with a roadmap and a partnership; Medicare's AI gatekeeper posted first outcomes.

> **ASSESSMENT**
> The US removed mandatory AI disclosure from EHR certification in the same days that Medicare's pilot published the first numbers on AI gatekeeping; the WHO, Europe and India moved the other way. The week's divergence is in force, with dates attached.

FINDINGS OF THE WEEK, GRADED BY EVIDENCE

POLICY HTI-5 deletes the model card

The final rule removes the decision-support source-attribute disclosures - the closest US regulation came to a mandatory model card - and 13 more criteria. HHS projects $1.55 billion in savings, its own figure. Disclosure moves from mandate to contract.

Why it matters: transparency is now a procurement question, not a certification one.

Source: ONC final rule overview

POLICY The WHO goes the other way - twice

Fifty-three European and Central Asian states unanimously adopted the first regional AI-in-health roadmap; a day later WHO's South-East Asia office signed a Wadhwani collaboration to test AI in India's public health system.

Why it matters: multilateral bodies are organising - a roadmap, an evaluation partnership - as Washington deletes a duty.

Sources: WHO Europe · WHO South-East Asia

RETROSPECTIVE Medicare's AI gatekeeper: service use down, decline fading

First outcomes of Medicare's WISeR prior-auth pilot (JAMA) tracked service use, not patient outcomes: targeted services fell 15-20% in pilot states, the decline attenuating over time; CMS records show vendor corrective-action plans.

Why it matters: whether it cut waste or care is still unknown.

Source: JAMA media release

BENCHMARK Five chatbots, one favourite drug brand

Five leading language models, 177,000 outputs each, converge on high-visibility drug brands; the leading product averaged 97.5% of first choices per model-disease pair.

Why it matters: what a model names is what a patient asks the doctor for.

Source: Nature Health

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EVIDENCE KEY

PROSPECTIVE	real patients, planned
FEASIBILITY	small, no comparison arm
BENCHMARK	tested on cases, not care
RETROSPECTIVE	past data, reviews
POLICY / PRESS	rules and reports
COMPANY	self-reported, unchecked

60%

of Americans are uncomfortable with AI in their own diagnosis - the public's terms, in one number

QUOTE OF THE WEEK

"HTI-5 Promotes Deregulatory Certification Actions, Leaves Information Blocking Rules Untouched."

HOLLAND & KNIGHT, ANALYSIS OF THE FINAL RULE

NEXT ISSUE · SUNDAY, 8 NOVEMBER 2026

EDITORIAL · OPINION

The rules diverged; the patient did not

BY DR. HEMANT L. PATEL

Washington deleted the closest thing US health-IT certification ever came to a mandatory model card. In the same days the WHO's European region adopted its first AI-in-health roadmap, and its South-East Asia office signed an evaluation partnership to test AI inside India's public-health system. Three instruments, one week, opposite directions.

The American change is real and dated: disclosure moves from certification duty to private contract. The European document is a shared direction, not a binding rule on any hospital. The India agreement is a testing partnership, not a law. Vendors will read all three. Buyers should not treat them as equal.

Beside the rulebook sits the week's only measured result on an AI gatekeeper. Medicare's WISeR pilot cut use of targeted services by 15–20 per cent in the first months; the drop later faded. The study tracked services, not patients. Whether it removed waste or care is still unknown. That is the number that matters more than any roadmap or deletion: a tool can change what gets done before anyone has shown it improved what happens to the person.

The public has already stated its terms. Sixty per cent of Americans are uncomfortable with AI in their own diagnosis. The models keep advancing; the measurement of what they do at the bedside does not. A rule that removes transparency, a roadmap that promises evaluation, and a pilot that counts services but not outcomes all leave the same gap.

The distance that still needs closing is not between Washington and Geneva. It is between the test set, the prior-auth log, and the patient.

WHAT TO WATCH

- A provider lawsuit or Congressional response to HTI-5 before the 28 December effective date.
- CMS's year-one call on WISeR - and any data on harm from forgone care.
- The AI-device lifecycle draft on the FDA's FY2027 agenda - and the Mosaic petition.
- WHO SEARO-Wadhwani evaluation protocols for India's public-health AI.
- Whether the Rajya Sabha demand for an AI-health law becomes a bill.
- Independent replication of the Doximity dosing-error rates.
- Whether India's sealed-room data pilots reach hospital data at scale.
- The MHRA's change-guidance draft, expected around December.
- Epic Connectors' first third-party AI integrations inside the EHR.

HOW THIS ISSUE WAS MADE

- Every item is drawn from the six daily editions of AI Health Times published 26-31 October 2026, re-checked against its primary source where one exists.
- Each item carries an evidence grade: PROSPECTIVE, FEASIBILITY, BENCHMARK, RETROSPECTIVE, POLICY, COMPANY or PRESS-REPORTED. Company-reported figures are labelled as such and are not audited outcomes.
- Facts appear once. Where a claim rests on a secondary report, the item says so.
- The week runs Sunday to Saturday; developments after Saturday evening wait for the next issue.

THE DEEP READ · STORY OF THE WEEK

A rule, a roadmap, a partnership

In the same seven days, Washington deleted binding disclosure duties from US health-record certification, the WHO's European region adopted a common roadmap, and its South-East Asia office signed an evaluation partnership - three different instruments, moving in opposite directions.

WHAT WASHINGTON DID

The HTI-5 final rule cuts 14 of 60 certification criteria for health IT and revises seven more. Among the deleted: the decision-support source-attribute disclosures - the requirement that certified software show what sits inside its predictive models, which the rule itself calls 'model cards' functioning as 'nutrition labels'. HHS projects $1.55 billion in present-value savings, its own estimate. These deletions take effect on 28 December; a few unrelated removals slip to 1 January 2027. Hospital groups warn they are losing the transparency they need to buy and deploy safely.

WHAT GENEVA AND DELHI DID

WHO Europe's member states - 53 of them, unanimously - adopted the region's first roadmap for AI in health, running to 2033, with governance, workforce and evaluation built in. A roadmap commits its signatories to a direction; it binds no hospital by itself. A day later the WHO's South-East Asia office signed a collaboration with LEHS's Wadhwani AI programme to design and evaluate AI in India's public health system: a testing partnership, not a regulation.

WHAT IS CONFIRMED

The US rule is final, dated and binding on certified software. The European roadmap is adopted; it binds no one by itself. The India collaboration is a signed evaluation partnership, not a law. One week, three instruments of very different force.

WHAT IS UNSETTLED

Whether HTI-5 survives contact with providers and Congress before 28 December. And whether contractual transparency happens once it is no longer mandatory - small hospitals buy what big vendors sell.

WHY IT MATTERS NOW

The instruments are not equivalent, but vendors read direction, not just law. Washington is stepping back from mandated disclosure just as Europe and India organise around evaluation. Product teams will plan for both signals: evaluation evidence will be expected somewhere, and in America it must now be secured by contract.

THE VIEW FROM INDIA

India is choosing the WHO path: a signed evaluation partnership, a parliamentary demand for an AI-in-healthcare law, and state capacity built ahead of the rules. The specifics are in the Radar, page 4.

Source: ONC: HTI-5 final rule overview
Source: Federal Register: HTI-5 final rule text
Source: WHO Regional Office for Europe

THE ARGUMENT IN ONE LINE

“When one regulator deletes a disclosure duty and fifty-three countries sign a common direction, the market does not converge - it chooses.”

MECHANISM OF ACTION

What ONC certification is

The Office of the National Coordinator certifies the health-record software used across US federal programmes. Its criteria define what certified software must do and show; removing a criterion removes a duty nearly everywhere American medicine keeps records.

What the model card was

The 2023 HTI-1 rule made certified systems expose 'source attributes' for predictive decision support: training-data traits, fairness and validity testing, monitoring. It was the closest US regulation came to a mandatory model card - and it is what HTI-5 removes.

Rule, guidance, paper

A final rule binds. Draft guidance advises. A discussion paper only asks. HTI-5 is the first kind; the FDA's promised AI-device document is the second. Reading them as one 'rulebook' is an editorial interpretation of different instruments, not equivalent legal regimes.

What replaces a mandate

Procurement. With certification silent, health systems must write disclosure, validation and monitoring duties into their own contracts. Transparency becomes a purchasing discipline - strong where buyers are sophisticated, absent where they are not.

THE RADAR · RUNNING STORIES AND INNOVATIONS WE TRACK

What we are following, and where it stands

Updated every Sunday. Each item carries a stage and a change flag: NEW ADVANCED UNCHANGED CLOSED . Flags show movement since the 25 October issue.

ITEM	WHAT IT IS	WHERE IT STANDS	WHAT DECIDES IT
STORY **US EHR certification rollback (HTI-5)** NEW	ONC certification rules for health-record software, including AI disclosure duties.	RULE FINAL HTI-5 removes 14 of 60 criteria; the source-attribute 'model card' disclosures are gone. HHS projects $1.55B savings; hospitals warn they lose buying transparency. Effective 28 December. Source: ONC final rule overview	Provider lawsuits or a Congressional response before the effective date.
STORY **Medicare's AI prior-auth pilot (WISeR)** NEW	AI-enabled prior authorization in six pilot states.	MEASURED First outcomes: targeted services fell 15-20% in pilot states, the decline attenuating over time; FOIA records show CMS corrective-action plans to vendors. Whether forgone care was waste or treatment is unknown. **Would change our view:** outcome data on denied-care harm. Source: JAMA media release	CMS's year-one continuation decision.
STORY **FDA generative-AI device policy** ADVANCED	The promised rule sketch for general-purpose medical AI.	DRAFTING The docket closed with 500-plus filings; the FY2027 agenda lists an AI-device lifecycle draft guidance; AdvaMed argues existing frameworks suffice. Source: In Compliance Magazine	The draft itself, and an answer to the Mosaic jurisdiction petition.
INNOVATION **Diagnostic-LLM benchmarks** ADVANCED	Generalist language models tested against diagnostic cases.	BENCHMARK After MAI-DxO's peer-reviewed Science result, a NEJM AI analysis priced LLM care plans at about 3x physician plans - the cost claim now cuts both ways. Source: NEJM AI	Prospective trials on real cohorts; audited cost figures.
STORY **Human oversight of clinical AI** ADVANCED	Whether human review catches wrong AI output, as deployment rules assume.	MEASURED Doximity: dosing errors in 2.3%+ of answers from nine general models, 19.5 per 1,000 serious - released beside its own verification product; replication owed. Pew: 60% of Americans uncomfortable with AI in their own care. Source: Healthcare Dive	Independent replication of the dosing-error rates.
STORY **Epic's AI ecosystem** ADVANCED	Draft replies, Connectors, third-party AI in the EHR.	DEPLOYING First large panel study of the draft-reply tool (983,000 responses): adopters saw 8.6% faster turnaround, 29.5% lower use; after-hours burden unchanged. Source: JAMA Network Open	Adoption curves as drafts default on; Connectors' first AI integrations.
STORY **AI data governance** ADVANCED	Who can see patient data when AI trains or answers.	BUILDING India piloted 'sealed rooms' - contracts execute in secure environments, data never leaves custody; Zocdoc opened agent booking behind registration and consent gates. Source: Mint	Whether sealed-room training reaches hospital data at scale.
INNOVATION **Supervised patient-facing AI** ADVANCED	Chatbots and agents that meet patients before or instead of a clinician.	DEPLOYING Zocdoc opened booking to AI agents; a lawsuit claims a chatbot advised on psychiatric drug tapering - allegations untested, coverage recap-level. Source: Healthcare Dive	Post-market safety data; consent standards for record access.
STORY **India: AI rules and capacity** ADVANCED	Regulation demands, institutions and skilling.	BUILDING A Rajya Sabha MP demanded a law for AI in healthcare and medical education; PGI Chandigarh's AI centre began work; FICCI-KPMG: 1.25 million health workers a year to skill. Source: Times of India	Whether the demand becomes a bill; what the PGI hub ships.
STORY **UK: AI Airlock and change guidance** UNCHANGED	The MHRA sandbox and the promised rules for changing AI devices.	OPEN Airlock phase-3 applications remain open; the change-guidance draft is expected around December. Source: GOV.UK	Whether Airlock publishes methods other regulators adopt.

RAPID ROUNDS, BY DOMAIN

REGULATION & POLICY

REPORT **AMA on liability:** the AMA advanced a framework in which liability for clinical AI follows control - whoever directs the tool carries the risk.

REPORT **Reading HTI-5:** Holland & Knight's analysis: deregulatory on certification, silent on information blocking - the fullest law-firm read of the final rule.

REPORT **FDA's FY2027 list:** the agency's proposed-guidance agenda includes an AI-enabled device lifecycle draft - a promise, not a schedule.

REPORT **Clearances:** two more imaging 510(k)s (FFR-IQ, NeuroDR) and BoneEnhance, the first machine-learning layer for Rivanna's ultrasound spine platform.

MARKET & FUNDING

The pattern: deployment news outnumbered funding news, and most figures stayed company-reported.

COMPANY **Cleveland Clinic-Dyania:** their trial-matching AI work went enterprise-wide and took a 2026 Gartner eye-on-innovation award.

COMPANY **Vera Health:** $28.5 million Series A for an AI-driven clinical search engine.

COMPANY **Anthropic:** published early drug-development results - company-reported, unaudited.

COMPANY **Eko:** the CORE 300, a $199 AI stethoscope, launched for the US market.

COMPANY **Google in Singapore:** AI wired into five layers of the city-state's health system - an Asia-Pacific first.

EVIDENCE & TRIALS

STUDY **Agentic oncologist:** a Nature Cancer study let an agentic LLM grade charts for immune-related adverse events - macro-F1 0.92 retrospective, 0.72-0.79 silent prospective; OR 1.45 (95% CI 1.01-2.08), at two cents a note.

STUDY **Lancet catch-up:** what happens when a conversational AI sits in the primary-care consult - the week's Lancet analysis, covered in Monday's daily.

STUDY **Oncology scribe:** an AI scribe for the consult-prep visit saves about three minutes (npj Digital Medicine).

STUDY **Penn's Percival:** a model trained on 400,000 CT scans flags disease nobody asked about - opportunistic screening; trade-press reported.

STUDY **Diagnostic odyssey:** a pre-diagnostic platform claims over 89% reliability across 300-plus rare diseases - the figure is the platform's own (HT Insight).

INDIA

REPORT **North Goa:** the district hospital ran its first AI-assisted knee replacement (Meril's MISSO).

REPORT **Apollo:** a 360-degree AI-assisted stroke-care programme launched in Hyderabad.

REPORT **Surgy Health:** the Bengaluru firm launched an AI-enabled hospital information system for India and the GCC.

REPORT **Mulk Technology:** an AI 'digital hospital' platform for India, launched this week.

REPORT **NAPCON 2026:** AI for TB and lung-cancer detection took the stage at the national chest conference.

REPORT **Summit talk:** the Times Network India Health Summit heard the enthusiasm-and-helmets line on health AI.

VITAL SIGNS

-15%
epidural injections under Medicare's AI prior-auth pilot (JAMA)

14.3%
of LLM outputs named a single drug product (Nature Health)

+8.6% / -29.5%
turnaround up, use down for Epic's AI draft replies (JAMA Netw Open)

60%
Americans uncomfortable with AI in their own care (Pew)

$1.55B
HHS-projected savings from HTI-5 (department figure)

1.25M/yr
health workers India must skill in digital health (FICCI-KPMG)

FOLLOW-UP: THE WEEK AHEAD

28 DECEMBER

HTI-5 takes effect; watch for a provider lawsuit or Congressional response before it lands.

COMING WEEKS

CMS's year-one call on WISeR; the AI-device lifecycle draft guidance on the FDA's FY2027 agenda.

8 NOVEMBER

Next weekly issue; the daily editions resume Monday.

ONGOING

WHO SEARO-Wadhwani evaluation protocols; Grace Cancer's readout remains outstanding; movement on India's AI-law demand.

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